

Eating Disorders, Inpatient Behavioral Health Level of Care, Adult

ORG: B-904-IP (BHG)

MCG Health
Behavioral Health
Care
28th Edition

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Admission Guidelines

- Admission to **Inpatient Level of Care for Eating Disorder for Adult** (LOCUS Level 6 - Medically Managed Residential Services, Composite Score 28 or more) is indicated due to **ALL** of the following^{[A][B][C][D](1)(3)(12)(13)(14)(15)(16)}:
 - Patient risk and clinical condition are appropriate for inpatient treatment, as indicated by **1 or more** of the following:
 - ☐ Subnormal BMI or low expected body weight for height, age, and sex, and need for medical treatment of unstable physical condition and urgent refeeding are present, as indicated by **1 or more** of the following^{[E](17)(18)}  BMI Calculator:
 - Current rapid rate of weight loss that has created an unstable physical condition⁽³⁾
 - Core body temperature less than 97 degrees F (36.1 degrees C)⁽³⁾
 - Dehydration that is severe or persistent
 - Heart rate less than 40 beats per minute
 - Hypotension
 - Orthostatic hypotension not responsive to appropriate outpatient treatment (eg, hydration)
 - Prolonged corrected QT interval^[F]
 - Severe muscle weakness^[G]
 - Serum phosphorus less than 1.5 mg/dL (0.48 mmol/L)^[H]
 - Electrolyte abnormality that cannot be corrected (to near normal) in emergency department or other ambulatory setting (eg, serum potassium less than 2.5 mEq/L (mmol/L), serum sodium less than 130 mEq/L (mmol/L))
 - Significant injury due to purging (eg, mucosal (Mallory-Weiss) tear, hematemesis due to ongoing frequent vomiting, colonic injury due to enema misuse)
 - Malnutrition-related severe organ dysfunction or damage findings (eg, heart failure, arrhythmia, altered mental status)
 - Imminent risk of developing significant medical instability (eg, marked vital sign abnormalities, malnourishment requiring refeeding) due to rapid rate of weight loss⁽³⁾⁽¹⁷⁾
 - ☐ Supervisory needs, motivation to recover, weight-related behaviors, and comorbidities are appropriate for inpatient treatment, as indicated by **ALL** of the following:
 - Strict staff supervision of meals (may include monitoring of specialized feeding modality, such as nasogastric tube) and bathroom use (direct monitoring in bathroom) is necessary.⁽²¹⁾
 - Motivation to recover is very poor to poor (patient condition requires involuntary treatment, or if voluntary patient, highly structured, inpatient setting is necessary for adherence to care).^{[(21)](21)}
 - ☐ Behaviors or clinical findings (eg, weight gain pattern, food refusal, purging, medication use for weight control) are appropriate for inpatient level of care, as indicated by **1 or more** of the following^{[J](21)}:

- There has been sustained inability to achieve or maintain clinically appropriate weight goals.
- There has been continued or renewed compensatory weight-loss behavior (eg, food refusal, self-induced vomiting, or excessive exercise).[K][L]
- There has been continued or renewed use of pharmaceuticals with intent to control weight (eg, laxatives, diuretics, stimulants, or over-the-counter weight loss preparations).[K]

☐ Treatment services available at proposed level of care are necessary to meet patient needs and **1 or more** of the following[M] (15)(16):

- Specific condition related to admission diagnosis is present and judged likely to further improve at proposed level of care.
- Specific condition related to admission diagnosis is present and judged likely to deteriorate in absence of treatment at proposed level of care.
- Patient is receiving continuing care (eg, transition of care from less intensive level of care).

☐ Situation and expectations are appropriate for inpatient care for adult, as indicated by **1 or more** of the following(2)(6)(22)(23) (24):

- Patient is unwilling to participate in treatment voluntarily and requires treatment (eg, legal commitment) in an involuntary unit.
- Voluntary treatment at lower level of care is not feasible (eg, very short-term crisis intervention or residential care unavailable or insufficient for patient condition).
- Need for physical restraint, seclusion, or other involuntary treatment intervention is present (eg, actively violent patient for whom treatment in an involuntary unit is deemed necessary in accordance with applicable medical and legal criteria).
- Around-the-clock medical and nursing care to address symptoms and initiate intervention is required; specific need is identified.[N]
- Patient management/treatment at lower level of care is not feasible or is inappropriate (eg, less intensive level of care is unavailable or not suitable for patient condition or treatment history).
- Biopsychosocial stressors[O] potentially contributing to clinical presentation (eg, comorbidities,[P][Q] illness history, environment,[R] social network, ability to cope, and level of engagement[S]) have been assessed and are absent or manageable at proposed level of care.(2)(4)(6)(7)(8)(9)(11)(29)(31)(32)

Recovery Course

Stage	Clinical Status	Interventions	Evaluation
1	• Clinical Indications met[T] • Treatment plan with goals and progress measurement in place • Nutritional plan with measurable goals in place • Social Determinants of Health Assessment • Begin Discharge planning	• Appropriate crisis management instituted or documented as not needed • Medical treatment of physiologic abnormalities or injury ongoing or not needed • Begin Care coordination • Close observation or regular checks done for meals, bathroom use, and safety • Appropriate treatment plan review	• Evaluation completed and reviewed • Nutritional assessment completed • Symptom and functioning assessment at appropriate frequency documented • Physician evaluation of patient and progress at appropriate frequency (eg, daily) documented • Review of need for medication or adjustment at appropriate frequency documented
2	• Adequate adherence to dietary plan for next level of care • Purging, bingeing, and other problem behavior absent or manageable/treatable at available lower level of care • Patient able to function and participate at next level of care • Weight status acceptable • Risk status acceptable • Treatment goals for level of care met • Social Determinants of Health Assessment • Complete Discharge planning • Discharge	• Medical needs absent or manageable/treatable at available lower level of care	

(1)(2)(3)(12)(23)(33)(34)

Recovery Milestones are indicated in **bold**.

Care Planning and Evaluation

- Planning and evaluating appropriate care should address(3)(12)(22)(35)(36):
 - Evaluation and care needs, which may include(3)(12):
 - Treatments and procedures, including(1)(35)(37):
 - Psychosocial interventions emphasizing admission precipitants and barriers to discharge
 - Clinical management and psychoeducation
 - Psychological testing
 - Possible adjunctive medication(38)
 - Dietary planning and nutritional counseling(39)
 - Structured individual or family psychotherapy(40)
 - NG feeding or parenteral nutrition(41)
 - Physical monitoring (eg, primary care)(42)
 - Consultation, assessment, and other services, including:
 - Substance use disorder assessment
 - Self-help or support group referral
 - Social services consultation for placement or housing
 - Telehealth services if indicated^[U](43)

Discharge Guidelines

- Continued inpatient care generally is needed until **1 or more** of the following(3)(12)(15):
 - Inpatient care is no longer necessary due to adequate patient stabilization or improvement, as indicated by **ALL** of the following:
 - Adequate adherence to dietary plan for next level of care
 - Purging, bingeing, and other problem behavior absent or manageable/treatable at available lower level of care, as indicated by **ALL** of the following:
 - Purging (eg, self-induced vomiting), bingeing, and other problem behavior (eg, excessive exercise) absent or manageable/treatable
 - Patient able to function and participate at available lower level of care
 - Patient able to function and participate at next level of care
 - Weight status acceptable, as indicated by **1 or more** of the following(3):
 - Weight is increasing at rate of at least 1 lb (0.5 kg) per week in residential setting.
 - Weight is increasing at rate of at least 2 lb (0.9 kg) per week in hospital setting.
 - Weight is stable, and reason for admission was not absence of recent weight gain.
 - Risk status acceptable, as indicated by **ALL** of the following(2)(4)(7)(9)(29)(44):
 - Danger to self or others manageable/treatable, as indicated by **1 or more** of the following:
 - Absence of Thoughts of suicide, homicide, or serious Harm to self or to another
 - Thoughts of suicide, homicide, or serious Harm to self or to another present but manageable/treatable at available lower level of care
 - Patient and supports understand follow-up treatment and crisis plan.
 - Provider and supports are sufficiently available at lower level of care.
 - Patient, as appropriate, can participate as needed in monitoring at available lower level of care.
 - Medical needs absent or manageable/treatable at available lower level of care, as indicated by **ALL** of the following(2)(4)(7)(9)(29)(44):
 - Adverse medication effects absent or manageable/treatable
 - Medical comorbidity absent or manageable/treatable
 - Medical complications absent or manageable/treatable (eg, complications of eating disorder)
 - Substance-related disorder absent or manageable/treatable
 - Treatment goals for level of care met
 - Inpatient care is no longer appropriate as patient or guardian no longer consents to treatment and involuntary treatment is not deemed necessary.(45)(46)

Discharge Planning

- Discharge planning needs may include(3)(12):
 - Rapid decision and planning regarding next level of care; considerations include:
 - Plan for monitoring meals, if necessary
 - Plan for monitoring for dangerous ideation or behavior, if necessary(47)

- Aftercare plan development with input from multiple providers, patient, and patient's supports
- Preparation of patient and supports for transition to next level of care, including:
 - Ensure sufficient knowledge of:
 - Patient's illness
 - Dietary plan
 - Warning signs of relapse or illness exacerbation(48)
 - Treatment resources
 - Community supports
 - Assess ability to work and participate in usual activities.
 - Review crisis plan with patient and supports.(49)
- Follow-up appointments, including:
 - Structured individual or family psychotherapy(50)(51)(52)
 - Pharmacotherapy and clinical management(53)
 - Psychiatrist for psychiatric comorbidity
 - Medical care visit (eg, primary care) for medical comorbidity
 - Registered dietitian
- Referrals for community assistance and support, including:
 - Self-help or support groups for patient, family, and caregivers
 - Community services for housing, financial, food, or transportation needs
- Medications and supplies, including:
 - Psychotropic medications(38)
 - Medications for comorbid medical conditions
 - Vitamin and mineral supplements(54)
 - Liquid feeding supplements

References

1. Call CC, Attia E, Walsh BT. Feeding and eating disorders. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2065-2082. [Context Link 1, 2, 3, 4]
2. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11] [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16, 17, 18]
3. Crone C, et al. The American Psychiatric Association Practice Guideline for the Treatment of Patients With Eating Disorders. 4th ed [Internet] American Psychiatric Association. 2023 Feb Accessed at: http://www.psych.org/psych_pract/treatg/pg/prac_guide.cfm. [accessed 2023 Sep 19] [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12, 13, 14, 15, 16]
4. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08] [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12]
5. Level of care placement. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:174-306. [Context Link 1, 2]
6. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760. [Context Link 1, 2, 3, 4, 5, 6]
7. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25] [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10, 11, 12]
8. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct. [Context Link 1, 2, 3, 4, 5, 6, 7, 8, 9, 10]
9. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep. [Context Link 1, 2, 3, 4, 5, 6]
10. Assessment measures. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:841-857. [Context Link 1]
11. Rosser J, Michael S. Partial Hospitalization Programs and Intensive Outpatient Programs. 2021 AABH Standards and Guidelines [Internet] Association for Ambulatory Behavioral Healthcare. 2021 Accessed at: <https://aabh.org/>. [accessed 2023 Sep 12] [Context Link 1, 2]
12. Eating Disorders: Recognition and Treatment. NICE Guidance NG69 [Internet] National Institute for Health and Care Excellence. 2020 Dec Accessed at: <https://www.nice.org.uk/guidance>. [accessed 2023 Aug 08] [Context Link 1, 2, 3, 4, 5, 6, 7]
13. Feeding and eating disorders. In: American Psychiatric Association, editor. Diagnostic and Statistical Manual of Mental Disorders. DSM-5-TR ed. American Psychiatric Association; 2022:371-397. [Context Link 1]
14. Suarez-Pinilla P, et al. Inpatient treatment for anorexia nervosa: a systematic review of randomized controlled trials. Journal of Psychiatric Practice 2015;21(1):49-59. DOI: 10.1097/01.pra.0000460621.95181.e2. [Context Link 1] View abstract...

15. Madden S, Hay P, Touyz S. Systematic review of evidence for different treatment settings in anorexia nervosa. *World Journal of Psychiatry* 2015;5(1):147-153. DOI: 10.5498/wjp.v5.i1.147. [Context Link 1, 2, 3] View abstract...
16. Clausen L, Jones A. A systematic review of the frequency, duration, type and effect of involuntary treatment for people with anorexia nervosa, and an analysis of patient characteristics. *Journal of Eating Disorders* 2014;2(1):29. DOI: 10.1186/s40337-014-0029-8. [Context Link 1, 2] View abstract...
17. Hilbert A, Hoek HW, Schmidt R. Evidence-based clinical guidelines for eating disorders: international comparison. *Current Opinion in Psychiatry* 2017;30(6):423-437. DOI: 10.1097/YCO.0000000000000360. [Context Link 1, 2, 3] View abstract...
18. Mehler PS, Brown C. Anorexia nervosa - medical complications. *Journal of Eating Disorders* 2015;3:11. DOI: 10.1186/s40337-015-0040-8. [Context Link 1] View abstract...
19. Sachs KV, Harnke B, Mehler PS, Krantz MJ. Cardiovascular complications of anorexia nervosa: A systematic review. *International Journal of Eating Disorders* 2016;49(3):238-48. DOI: 10.1002/eat.22481. [Context Link 1] View abstract...
20. Sachs K, Andersen D, Sommer J, Winkelman A, Mehler PS. Avoiding medical complications during the refeeding of patients with anorexia nervosa. *Eating Disorders* 2015;1-11. DOI: 10.1080/10640266.2014.1000111. [Context Link 1] View abstract...
21. Levels of Care. [Internet] National Eating Disorders Association (NEDA). Accessed at: <https://www.nationaleatingdisorders.org/types-treatment>. Updated 2022 [accessed 2023 Oct 23] [Context Link 1, 2, 3]
22. Matorin AA, Shah AA, Ruiz P. Clinical manifestations of psychiatric disorders. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:1114-1150. [Context Link 1, 2]
23. Baron DA, Cobb RT, Juarez GM. Other psychiatric emergencies. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2622-2637. [Context Link 1, 2]
24. Winkler D, et al. Intensive care in psychiatry. *European Psychiatry* 2011;26(4):260-264. DOI: 10.1016/j.eurpsy.2010.10.008. [Context Link 1] View abstract...
25. Abadi M, et al. Achieving whole health: a preliminary study of TCMLH, a group-based program promoting self-care and empowerment among Veterans. *Health Education & Behavior* 2022;49(2):347-357. DOI: 10.1177/10901981211011043. [Context Link 1, 2, 3] View abstract...
26. Joshi H, Corcoran-Lozano P. The behavioral health consultant: roles and responsibilities. *Pediatric Clinics of North America* 2021;68(3):563-571. DOI: 10.1016/j.pcl.2021.02.002. [Context Link 1, 2, 3] View abstract...
27. Rosenbaum E, Gordon AE, Cresta J, Shaughnessy AF, Jonas WB. Implementing whole person primary care. *Annals of Family Medicine* 2023;21(2):Online. DOI: 10.1370/afm.2952. [Context Link 1, 2, 3] View abstract...
28. Pourat N, et al. Final Evaluation of California's Whole Person Care (WPC) Program. [Internet] UCLA Center for Health Policy Research. 2022 Dec Accessed at: <https://healthpolicy.ucla.edu/publications/Pages/default.aspx>. [accessed 2023 Sep 08] [Context Link 1, 2, 3]
29. Dimensional admission criteria and algorithm. In: Waller RC, et al., editors. *The ASAM Criteria: Treatment Criteria for Addictive, Substance-Related, and Co-occurring Conditions, Volume 1: Adults*. 4th ed. Hazelden Publishing; 2023. [Context Link 1, 2, 3, 4, 5, 6, 7, 8]
30. Continuum of care. In: Waller RC, et al., editors. *The ASAM Criteria: Treatment Criteria for Addictive, Substance-Related, and Co-occurring Conditions, Volume 1: Adults*. 4th ed. Hazelden Publishing; 2023. [Context Link 1]
31. Fong T. Assessment. In: Brady KT, Levin FR, Galanter M, Kleber HD, editors. *Textbook of Substance Use Disorder Treatment*. 6th ed. American Psychiatric Association Publishing; 2021:63-76. [Context Link 1]
32. Kober D, Martin A. Inpatient psychiatric, partial hospital, and residential treatment for children and adolescents. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:3797-3803. [Context Link 1]
33. Sudak HS. Suicide treatment. In: Sadock BJ, Sadock VA, Ruiz P, editors. *Kaplan and Sadock's Comprehensive Textbook of Psychiatry*. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2618-2622. [Context Link 1]
34. Barnhill JW. The psychiatric interview and mental status examination. In: Roberts LW, editor. *The American Psychiatric Publishing Textbook of Psychiatry*. 7th ed. Washington DC: American Psychiatric Association Publishing; 2019:3-30. [Context Link 1]
35. Monteleone AM, et al. Treatment of eating disorders: A systematic meta-review of meta-analyses and network meta-analyses. *Neuroscience and Biobehavioral Reviews* 2022;142:Online. DOI: 10.1016/j.neubiorev.2022.104857. [Context Link 1, 2] View abstract...
36. Treasure J, Duarte TA, Schmidt U. Eating disorders. *Lancet* 2020;395(10227):899-911. DOI: 10.1016/S0140-6736(20)30059-3. [Context Link 1] View abstract...
37. Himmerich H, et al. World Federation of Societies of Biological Psychiatry (WFSBP) guidelines update 2023 on the pharmacological treatment of eating disorders. *World Journal of Biological Psychiatry* 2023;1-64. DOI: 10.1080/15622975.2023.2179663. [Context Link 1] View abstract...
38. Han R, Bian Q, Chen H. Effectiveness of olanzapine in the treatment of anorexia nervosa: A systematic review and meta-analysis. *Brain and Behavior* 2022;12(2):Online. DOI: 10.1002/brb3.2498. [Context Link 1, 2] View abstract...
39. Ozier AD, Henry BW, American Dietetic Association. Position of the American Dietetic Association: nutrition intervention in the treatment of eating disorders. *Journal of the American Dietetic Association* 2011;111(8):1236-1241. DOI: 10.1016/j.jada.2011.06.016. [Context Link 1] View abstract...
40. Hay PJ, Claudino AM, Touyz S, Abd Elbaky G. Individual psychological therapy in the outpatient treatment of adults with anorexia nervosa. *Cochrane Database of Systematic Reviews* 2015, (verified by Cochrane 2018 Feb), Issue 7. Art. No.: CD003909. DOI: 10.1002/14651858.CD003909.pub2. [Context Link 1] View abstract...
41. Bendall C, Taylor NF. The effect of oral refeeding compared with nasogastric refeeding on the quality of care for patients hospitalised with an eating disorder: A systematic review. *Nutrition & Dietetics* 2023;80(1):44-54. DOI: 10.1111/1747-0080.12770. [Context Link 1] View abstract...

42. Emergencies in Eating Disorders: Guidance on Recognition and Management. (Replacing MARSIPAN and Junior MARSIPAN) [Internet] Royal College of Psychiatrists. College Report CR233; 2023 Jul Accessed at: <https://www.rcpsych.ac.uk/improving-care/campaigning-for-better-mental-health-policy/college-reports/2022-college-reports/cr233>. [created 2022 May; accessed 2023 Oct 20] [Context Link 1]
43. Brunt TJ, Gale-Grant O. Telepsychiatry: what clinicians need to know about digital mental healthcare. *BJPsych Advances* 2023;29(4):230-238. DOI: 10.1192/bja.2022.42. [Context Link 1, 2] View abstract...
44. Matching multidimensional severity and level of function with type and intensity of service. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. *ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions*. 3rd ed. Carson City, NV: The Change Companies; 2013:69-104. [Context Link 1, 2]
45. Gorrell S, et al. Understanding non-routine discharge: Factors that are associated with premature termination from higher levels of care in adults with anorexia nervosa. *Eating Disorders* 2022;30(6):686-699. DOI: 10.1080/10640266.2021.2011648. [Context Link 1] View abstract...
46. Mac Donald B, Bulik CM, Larsen JT, Carlsen AH, Clausen L, Petersen LV. Involuntary treatment in patients with anorexia nervosa: utilization patterns and associated factors. *Psychological Medicine* 2023;53(5):1999-2007. DOI: 10.1017/S003329172100372X. [Context Link 1] View abstract...
47. Cliffe C, Shetty H, Himmerich H, Schmidt U, Stewart R, Dutta R. Suicide attempts requiring hospitalization in patients with eating disorders: A retrospective cohort study. *International Journal of Eating Disorders* 2020;53(5):458-465. DOI: 10.1002/eat.23240. [Context Link 1] View abstract...
48. Gorrell S, Hail L, Reilly EE. Predictors of treatment outcome in eating disorders: a roadmap to inform future research efforts. *Current Psychiatry Reports* 2023;25(5):213-222. DOI: 10.1007/s11920-023-01416-w. [Context Link 1] View abstract...
49. Bryan CJ, et al. Effect of crisis response planning vs. contracts for safety on suicide risk in U.S. Army Soldiers: A randomized clinical trial. *Journal of Affective Disorders* 2017;212:64-72. DOI: 10.1016/j.jad.2017.01.028. [Context Link 1] View abstract...
50. Kass AE, Kolko RP, Wilfley DE. Psychological treatments for eating disorders. *Current Opinion in Psychiatry* 2013;26(6):549-55. DOI: 10.1097/YCO.0b013e328365a30e. [Context Link 1] View abstract...
51. Pisetsky EM, Schaefer LM, Wonderlich SA, Peterson CB. Emerging psychological treatments in eating disorders. *Psychiatric Clinics of North America* 2019;42(2):219-229. DOI: 10.1016/j.psc.2019.01.005. [Context Link 1] View abstract...
52. Fisher CA, Skocic S, Rutherford KA, Hetrick SE. Family therapy approaches for anorexia nervosa. *Cochrane Database of Systematic Reviews* 2019, Issue 5. Art. No.: CD004780. DOI: 10.1002/14651858.CD004780.pub4. [Context Link 1] View abstract...
53. Muratore AF, Attia E. Psychopharmacologic management of eating disorders. *Current Psychiatry Reports* 2022;24(7):345-351. DOI: 10.1007/s11920-022-01340-5. [Context Link 1] View abstract...
54. Marzola E, Nasser JA, Hashim SA, Shih PA, Kaye WH. Nutritional rehabilitation in anorexia nervosa: review of the literature and implications for treatment. *BMC Psychiatry* 2013;13:290. DOI: 10.1186/1471-244X-13-290. [Context Link 1] View abstract...

Footnotes

[A] Inpatient psychiatric units generally are locked, equipped to restrain or seclude patients for safety if necessary, and staffed by nurses around the clock. A covering physician is available to assess patients if indicated around the clock.(1)(2) [A in Context Link 1]

[B] Whether or not a patient with an eating disorder should be hospitalized on a psychiatric vs a general medical unit depends on a variety of factors, including medical and psychiatric status, skills and abilities of staff in the proposed admitting institution, and the availability of suitable programs to address medical and psychiatric needs. There is some evidence to suggest that when hospital admission is necessary, admission to inpatient units specializing in eating disorder treatment may be associated with better outcomes. (3) [B in Context Link 1]

[C] The purpose of the care guidelines is to promote evidence-based care across the continuum of care to enhance the delivery of quality healthcare. Indications are presented for different levels of care. These indications help define the optimal level of care and can assist in developing alternatives to higher levels of care, tracking patient progress during treatment within a level of care, facilitating the progress of patients whose recovery is delayed, and preparing comprehensive plans for transition of patients from one level of care to another. Relevant professional society guidelines are foundational content for evaluation and treatment of behavioral health disorders at different levels of care and are complemented by the best available published evidence.(2)(4)(5)(6)(7)(8)(9)(10)(11) [C in Context Link 1]

[D] Composite score does not replace clinical judgment and is meant as a guide to assist in determining needed level of care. Extreme risk of harm, severe functional impairment, or severe comorbidity may independently necessitate placement at inpatient level of care.(2) [D in Context Link 1]

[E] Practice guidelines for the treatment of patients with eating disorders indicate that the decision for hospitalization of a patient with an eating disorder should be made on an individualized basis, utilizing a patient's personal weight status and risk of developing medical complications, instead of using standardized cutoff levels for BMI, rate of weight change, or percentage of healthy body weight.(17) [E in Context Link 1]

[F] QT-interval prolongation is a possible complication of anorexia nervosa and a suspected contributor to the increased incidence of sudden death observed in patients with this disorder.(19) [F in Context Link 1]

[G] Muscle weakness is considered severe if it prevents the patient from rising from a seated position without use of the arms, or rising from a supine position even with use of the arms, or from being able to sit up at all.(12) [G in Context Link 1]

[H] Total body phosphorus depletion, a possible consequence of malnutrition, increases the risk (during nutritional rehabilitation) of refeeding syndrome, a condition involving severe alterations in fluid and electrolyte balances that can lead to potentially life-threatening cardiac and neuromuscular complications.(20) [H in Context Link 1]

[I] A practice guideline for the treatment of patients with eating disorders indicates that an assessment of motivation to recover is indicated in the initial admission assessment to assist in determining the appropriate level of care to treat the eating disorder.(3) [I in Context Link 1]

[J] Patients appropriate for inpatient eating disorder treatment may be preoccupied with intrusive or repetitive thoughts for more than 6 hours a day.(3) [J in Context Link 1]

[K] A practice guideline for the treatment of patients with eating disorders indicates that patients engaged in and unable to control multiple daily episodes of purging behaviors that are severe, persistent, and disabling, and who have been unable to regulate these behaviors despite appropriate trials of outpatient care may be considered for inpatient management. This is the case even in the absence of metabolic abnormalities, provided that they otherwise meet inpatient admission criteria related to supervisory needs, motivation, and comorbidities.(3) [K in Context Link 1, 2]

[L] A practice guideline for the treatment of patients with eating disorders indicates that compulsive exercising rarely is a sole indication for increasing level of care.(3) [L in Context Link 1]

[M] Assessment of the likelihood of benefit at the proposed level of care may help identify subgroups of patients more likely to respond to particular treatments and help optimize the care plan to increase the likelihood of successful recovery. It may also help determine if a specialized treatment setting (eg, dual-diagnosis program) or a different level of service intensity would be more appropriate to address the patient's needs.(2)(3)(5) [M in Context Link 1]

[N] Examples of medical conditions appropriate for inpatient care include conditions that require intensive, around-the-clock medical monitoring and daily nursing interventions, or patients with significant metabolic or ECG abnormalities related to vomiting.(2) [N in Context Link 1]

[O] Biopsychosocial stressors may impact the level of care necessary to manage a psychiatric or behavioral condition, including the ability of the program to meet comprehensive patient needs, ensure treatment adherence, enhance motivation, or prevent relapse (ie, comorbidities, environmental factors, or other barriers may prevent effective treatment at a less intensive level of care than might otherwise be appropriate to the patient's condition). Biopsychosocial assessment factors should be incorporated into care planning, including planned treatment goals, and intensity and duration of interventions. A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(25)(26)(27)(28) Any identified deficits should be manageable by the program directly or through alternative arrangements.(2)(4)(7)(8) [O in Context Link 1]

[P] Comorbid conditions may directly impact the experience of psychiatric symptoms (eg, COPD and anxiety), or may indirectly impact determination of the appropriate venue for care (eg, routine blood sugar and insulin management in people with diabetes). If clinically appropriate, testing related to diagnosis or management (eg, screening for liver and kidney function, hepatitis, HIV, syphilis, tuberculosis) may be performed off-site.(2)(4)(7)(8) Assessment and treatment of co-occurring medical and/or developmental conditions through services and treatment settings capable of rendering integrated care is recommended.(2)(4)(6)(7)(8)(9) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(25)(26)(27)(28) The level that comorbid medical and/or developmental conditions are present may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care).(2)(4)(7)(8)(29) [P in Context Link 1]

[Q] Evaluation/assessment and treatment of co-occurring substance use disorders through services and treatment settings capable of rendering integrated care is recommended.(2)(4)(6)(7)(8) A "whole person care" approach that coordinates care across physical health, behavioral health, and social service sectors may bridge the gap of physical and mental health.(25)(26)(27)(28) The level that comorbid substance use disorders contribute to the primary presenting condition may be described on a continuum (none/absent, low, moderate, and severe) and may impact determination of the appropriate level of care for treatment (ie, admission to a higher level of care or specialty dual-diagnosis program).(2)(4)(7)(8)(29) [Q in Context Link 1]

[R] The degree of environmental stressors and amount of support in the patient recovery environment should be considered in the context of the clinical presentation in determining the appropriate level of care for treatment. The level of environmental stressors may be low, mild, moderate, high, or extreme. The level of support in the environment may range from absent or minimal to limited to supportive or highly supportive.(2)(4)(6)(7)(8)(9)(29) [R in Context Link 1]

[S] Participation motivated by a wish to avoid negative consequences rather than accept the need to work toward recovery may require more intense monitoring and follow-up.(29)(30) The patient's level of engagement may be described on a continuum: optimal, positive,

limited, minimal, or unengaged. Readiness to change may range from actively and willingly engaged to unable to follow treatment recommendations due to clinical condition.(2)(4)(7)(8)(29) [S in Context Link 1]

[T] See Admission Guidelines in this guideline. [T in Context Link 1]

[U] Telehealth may improve access to and coordination of mental health care. Patients with behavioral health diagnoses may use telehealth or telepsychiatry to access mental health support or care (eg, to get assistance with problem-solving techniques or ask about presenting symptoms or medication use).(43) [U in Context Link 1]

Definitions

Acute kidney injury (stage 2)

- Acute kidney injury (stage 2) with significant clinical findings, as indicated by **ALL** of the following(1)(2)(3):
 - Acute^[A] kidney injury (stage 2), as indicated by **1 or more** of the following^[B]:
 - Within past 7 days, rise in serum creatinine between 2 and 2.9 times its baseline value (increase of 3 times baseline would qualify as stage 3 acute kidney injury)
 - Decreased urine output,^[C] as indicated by **ALL** of the following:
 - Assessment of urine output appropriate (eg, adequate volume status, no urinary obstruction)
 - Urine output less than 0.5 mL/kg/hr for 12 hours
 - Significant clinical findings, as indicated by **1 or more** of the following^[B]:
 - Hemodynamic instability
 - Worsening clinical status despite observation care (eg, rising creatinine or urine output remains less than 0.5 mL/kg/hr)
 - Altered mental status
 - Cardiac arrhythmias of immediate concern
 - Severe hypertension (SBP greater than 180 mm Hg or DBP greater than 120 mm Hg in adults or greater than the 95th percentile for age, gender, and height plus 30 mm Hg in pediatric patients)(4)(5)
 - Significant respiratory findings (eg, pulmonary edema) that are severe (eg, Hypoxemia) or persist despite observation care
 - Clinically significant electrolyte abnormality that is severe (eg, hyperkalemia with severe ECG findings)^[D] or persists despite observation care
 - Clinically significant metabolic abnormality (eg, metabolic acidosis) that is severe or persists despite observation care

References

1. Kidney Disease: Improving Global Outcomes (KDIGO) Acute Kidney Injury Work Group. KDIGO clinical practice guideline for acute kidney injury. *Kidney International. Supplement* 2012;2(1):1-138. (Reaffirmed 2023 May)
2. Weisbord SD, Palevsky PM. Prevention and management of acute kidney injury. In: Yu AS, Chertow GM, Luyckx VA, Marsden PA, Skorecki K, Taal MW, editors. *Brenner and Rector's The Kidney*. 11th ed. Philadelphia, PA: Elsevier; 2020:940-977.e15.
3. Palevsky PM, et al. KDOQI US commentary on the 2012 KDIGO clinical practice guideline for acute kidney injury. *American Journal of Kidney Diseases* 2013;61(5):649-72. DOI: 10.1053/j.ajkd.2013.02.349.
4. Whelton PK, et al. 2017 ACC/AHA/AAPA/ABC/ACPM/AGS/APhA/ASH/ASPC/NMA/PCNA Guideline for the prevention, detection, evaluation, and management of high blood pressure in adults: a report of the American College of Cardiology/American Heart Association Task Force on Clinical Practice Guidelines. *Circulation* 2018;138(17):e484-e594. DOI: 10.1161/CIR.0000000000000596. (Reaffirmed 2023 Jan)
5. Flynn JT, et al. Clinical practice guideline for screening and management of high blood pressure in children and adolescents. *Pediatrics* 2017;140(3):e20171904. DOI: 10.1542/peds.2017-1904. (Reaffirmed 2022 Aug)
6. Mirvis DM, Goldberger AL. Electrocardiography. In: Libby P, Bonow RO, Mann DL, Tomaselli GF, Bhatt DL, Solomon SD, editors. *Braunwald's Heart Disease: A Textbook of Cardiovascular Medicine*. 12th ed. Elsevier; 2022:141-174.e3.
7. Mount DB. Fluid and electrolyte disturbances. In: Loscalzo J, Fauci A, Kasper D, Hauser S, Longo D, Jameson JL, editors. *Harrison's Principles of Internal Medicine*. 21st ed. McGraw Hill Education; 2022:338-356.

Footnotes

- A. A specialty society practice guideline defines acute kidney injury as a change in renal function that is known or presumed to have occurred over the previous 7 days.(1)
- B. In determining the stage of acute kidney injury (eg, stage 2 vs stage 3), patients should be staged according to the criteria that give them the highest stage.(1)
- C. The use of urine output as an indicator of acute kidney injury is important, as change in urine output can occur before a noticeable rise in creatinine.(1)(3) It is also suggested that urine output can identify patients with acute kidney injury that may otherwise be missed.(1)(3) However, urine output as a metric is only valid under the appropriate conditions. For example, the patient must not be hypovolemic, there must not be urinary obstruction, and the collection and measurement of urinary output must be complete and accurate.(1)(3) Use of urinary output to define acute kidney injury can be inaccurate in obese patients due to the nonlinear relationship between body weight and urine output. For example, in a 100-kg (220-lb) patient, urine output of 45 mL/hour over 12 hours (adequate output) would qualify as stage 2 acute kidney injury.(1)(3)

- D. ECG findings include peaked T-waves, PR interval greater than 0.20 seconds, QRS interval greater than 0.12 seconds, and arrhythmia.(6)(7)

Acute renal failure (stage 3 acute kidney injury)

- Acute^[A] kidney injury (stage 3),^[B] as indicated by **1 or more** of the following^[C](1)(2)(3):
 - Within past 7 days, rise in serum creatinine to 3 times its baseline value or higher
 - Within past 7 days, rise in serum creatinine to at least 1.5 times its baseline value (increase of 50%) and serum creatinine is greater than 4 mg/dL (354 micromoles/L)
 - In child younger than 18 years, estimated glomerular filtration rate less than 35 mL/min/1.73m² (0.59 mL/sec/1.73m²)
 -  eGFR - Pediatric Calculator and **1 or more** of the following:
 - Within past 7 days, rise in serum creatinine to at least 1.5 times its baseline value (increase of 50%)
 - Within past 48 hours, rise in serum creatinine greater than 0.3 mg/dL (26.5 micromoles/L)
 - Decreased urine output,^[D] as indicated by **ALL** of the following:
 - Assessment of urine output appropriate (eg, adequate volume status, no urinary obstruction)
 - Oligoanuria, as indicated by **1 or more** of the following:
 - Urine output less than 0.3 mL/kg/hour for 24 hours
 - Anuria (urine output less than 0.1 mL/kg/hour) for 12 hours
 - Initiation of renal replacement therapy (RRT) required(4)

References

- Kidney Disease: Improving Global Outcomes (KDIGO) Acute Kidney Injury Work Group. KDIGO clinical practice guideline for acute kidney injury. *Kidney International*. Supplement 2012;2(1):1-138. (Reaffirmed 2023 May)
- Weisbord SD, Palevsky PM. Prevention and management of acute kidney injury. In: Yu AS, Chertow GM, Luyckx VA, Marsden PA, Skorecki K, Taal MW, editors. *Brenner and Rector's The Kidney*. 11th ed. Philadelphia, PA: Elsevier; 2020:940-977.e15.
- Palevsky PM, et al. KDOQI US commentary on the 2012 KDIGO clinical practice guideline for acute kidney injury. *American Journal of Kidney Diseases* 2013;61(5):649-72. DOI: 10.1053/j.ajkd.2013.02.349.
- Gaudry S, Palevsky PM, Dreyfuss D. Extracorporeal kidney-replacement therapy for acute kidney injury. *New England Journal of Medicine* 2022;386(10):964-975. DOI: 10.1056/NEJMra2104090.

Footnotes

- A. A specialty society practice guideline defines acute kidney injury as a change in renal function that is known or presumed to have occurred over the previous 7 days.(1)
- B. For purposes of these criteria, the term acute renal failure can be viewed as stage 3 acute kidney injury in the KDIGO classification system.(1) Of note, in the ICD-10 diagnostic coding system (main coding scheme used in labeling diagnoses), the relevant codes covering the clinical entity of significant acute kidney injury use the term acute kidney failure (eg, code N17.9).
- C. In determining the stage of acute kidney injury (eg, stage 2 vs stage 3), patients should be staged according to the criteria that give them the highest stage.(1)
- D. The use of urine output as an indicator of acute kidney injury is important, as change in urine output can occur before a noticeable rise in creatinine.(1)(3) It is also suggested that urine output can identify patients with acute kidney injury that may otherwise be missed.(1)(3) However, urine output as a metric is only valid under the appropriate conditions. For example, the patient must not be hypovolemic, there must not be urinary obstruction, and the collection and measurement of urinary output must be complete and accurate.(1)(3) Use of urinary output to define acute kidney injury can be inaccurate in obese patients due to the nonlinear relationship between body weight and urine output. For example, in a 100-kg (220-lb) patient, urine output of 45 mL/hour over 12 hours (adequate output) would qualify as stage 2 acute kidney injury.(1)(3)

Altered mental status

- Altered mental status (ie, different from baseline), as indicated by **1 or more** of the following(1)(2)(3)(4):
 - Confusional state (eg, disorientation, difficulty following commands, deficit in attention)
 - Lethargy (eg, awake or arousable, but with drowsiness; reduced awareness of self and environment)
 - Obtundation (ie, arousable only with strong stimuli, lessened interest in environment, slowed responses to stimulation)
 - Stupor (ie, may be arousable but patient does not return to normal baseline level of awareness)
 - Coma (ie, not arousable)

References

- Maher PJ. Confusion. In: Walls RM, editor. *Rosen's Emergency Medicine: Concepts and Clinical Practice*. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:126-131.e1.
- Lei C, Smith C. Depressed consciousness and coma. In: Walls RM, editor. *Rosen's Emergency Medicine: Concepts and Clinical Practice*. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:117-125.e1.

3. Abraham G, Maher PJ. Delirium and dementia. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:1269-1280.e2.
4. Schor NF. Neurologic evaluation. In: Kliegman RM, St. Geme JW, Blum NJ, Shah SS, Tasker RC, Wilson KM, editors. Nelson Textbook of Pediatrics. 21st ed. Philadelphia, PA: Elsevier; 2020:3053-3063.e1.

Appropriate treatment plan review

- Appropriate treatment plan review and outcome includes **1 or more** of the following(1):
 - Maintenance of current plan under which patient is making substantial progress
 - Review and modification based on inadequate progress
 - No review deemed needed as patient has progressed and is transitioning to new level of care plan

References

1. Geller JL. The role of the hospital in the care of persons with mental illness. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:4325-4346.

Cardiac arrhythmias of immediate concern

- Cardiac arrhythmias or findings of immediate concern, as indicated by **1 or more** of the following(1)(2)(3):
 - Heart rhythms that are inherently dangerous or unstable, as indicated by **1 or more** of the following(4)(5)(6):
 - Resuscitated ventricular fibrillation or cardiac arrest
 - Ventricular escape rhythm
 - Sustained ventricular tachycardia (30 seconds or more of ventricular rhythm at greater than 100 beats per minute)
 - Nonsustained ventricular tachycardia and **1 or more** of the following:
 - Suspected cardiac ischemia as cause or consequence of ventricular tachycardia
 - Acute myocarditis
 - Unstable cardiac conduction defects, as indicated by **1 or more** of the following(6)(7)(8):
 - Type II second-degree atrioventricular block
 - Third-degree atrioventricular block
 - New-onset left bundle branch block with suspected myocardial ischemia
 - Any heart rhythm and **1 or more** of the following(4)(5)(9)(10)(11):
 - Continuous long-term ECG monitoring needed (eg, initiation of drug requiring monitoring beyond observation care)
 - Patient has automatic implanted cardioverter-defibrillator that is repeatedly firing, malfunctioning, or in need of immediate adjustment of settings beyond scope of observation care.
 - Heart rhythms of concern due to **1 or more** of the following:
 - Hypotension
 - Respiratory distress
 - Association with other significant symptoms (eg, syncope)(9)(10)(12)

References

1. Curtis AB, Tomaselli GF. Approach to the patient with cardiac arrhythmias. In: Libby P, Bonow RO, Mann DL, Tomaselli GF, Bhatt DL, Solomon SD, editors. Braunwald's Heart Disease: A Textbook of Cardiovascular Medicine. 12th ed. Elsevier; 2022:1145-1162.e7.
2. Dalal AS, Van Hare GF. Disturbances of rate and rhythm of the heart. In: Kliegman RM, St. Geme JW, Blum NJ, Shah SS, Tasker RC, Wilson KM, editors. Nelson Textbook of Pediatrics. 21st ed. Philadelphia, PA: Elsevier; 2020:2436-2447.e1.
3. Joglar JA, et al. 2023 HRS Expert Consensus Statement on the management of arrhythmias during pregnancy. Heart Rhythm 2023;20(10):Online. DOI: 10.1016/j.hrthm.2023.05.017.
4. El-Chami M, Fernando L. Ventricular arrhythmias. In: McKean SC, Ross JJ, Dressler DD, Scheurer DB, editors. Principles and Practice of Hospital Medicine. 2nd ed. New York, NY: McGraw-Hill Education; 2017:1003-1014.
5. Al-Khatib SM, et al. 2017 AHA/ACC/HRS guideline for management of patients with ventricular arrhythmias and the prevention of sudden cardiac death: a report of the American College of Cardiology/American Heart Association Task Force on Clinical Practice Guidelines and the Heart Rhythm Society. Circulation 2018;138(13):e272-e391. DOI: 10.1161/CIR.0000000000000549. (Reaffirmed 2023 Jul)
6. Epstein AE, et al. 2012 ACCF/AHA/HRS focused update incorporated into the ACCF/AHA/HRS 2008 guidelines for device-based therapy of cardiac rhythm abnormalities: a report of the American College of Cardiology Foundation/American Heart Association Task Force on Practice Guidelines and the Heart Rhythm Society. Circulation 2013;127(3):e283-e352. DOI: 10.1161/CIR.0b013e318276ce9b. (Reaffirmed 2022 Jun)
7. Miller JM, Ellenbogen KA. Therapy for cardiac arrhythmias. In: Libby P, Bonow RO, Mann DL, Tomaselli GF, Bhatt DL, Solomon SD, editors. Braunwald's Heart Disease: A Textbook of Cardiovascular Medicine. 12th ed. Elsevier; 2022:1208-1244.
8. Park DS, Fishman GI. The cardiac conduction system. Circulation 2011;123(8):904-15. DOI: 10.1161/CIRCULATIONAHA.110.942284.
9. Joglar JA, et al. 2023 ACC/AHA/ACCP/HRS Guideline for the diagnosis and management of atrial fibrillation: a report of the American College of Cardiology/American Heart Association joint committee on clinical practice guidelines. Journal of the American

College of Cardiology 2023 Nov;online. DOI: 10.1016/j.jacc.2023.08.017. (Reaffirmed 2023 Nov 30)

10. Chun EB, McGorisk GM. Supraventricular tachyarrhythmias. In: McKean SC, Ross JJ, Dressler DD, Scheurer DB, editors. Principles and Practice of Hospital Medicine. 2nd ed. New York, NY: McGraw-Hill Education; 2017:980-995.
11. Hoskins MH, De Lurgio DB. Pacemakers, defibrillators, and cardiac resynchronization devices in hospital medicine. In: McKean SC, Ross JJ, Dressler DD, Scheurer DB, editors. Principles and Practice of Hospital Medicine. 2nd ed. New York, NY: McGraw-Hill Education; 2017:1025-34.
12. Westerman S, Manogue M, Hoskins MH. Bradycardia. In: McKean SC, Ross JJ, Dressler DD, Scheurer DB, editors. Principles and Practice of Hospital Medicine. 2nd ed. New York, NY: McGraw-Hill Education; 2017:996-1002.

Care coordination

- Care coordination includes communication with **ALL** of the following(1)(2):
 - Outside psychiatrist, other therapists, and medical care providers as appropriate
 - Patient, family, guardian, and other supports as appropriate
 - All team members in other disciplines involved in ongoing patient care

References

1. Geller JL. The role of the hospital in the care of persons with mental illness. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:4325-4346.
2. Kober D, Martin A. Inpatient psychiatric, partial hospital, and residential treatment for children and adolescents. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:3797-3803.

Dehydration that is severe or persistent

- Dehydration that is severe or persistent, as indicated by **1 or more** of the following(1)(2)(3)(4):
 - Clinical findings of severe dehydration, as indicated by **1 or more** of the following:
 - Hemodynamic instability
 - Acute renal failure (stage 3 acute kidney injury)
 - Acute kidney injury (stage 2)
 - Serum sodium greater than 150 mEq/L (mmol/L)
 - Dehydration that is persistent, as indicated by **ALL** of the following:
 - Oral rehydration therapy not tolerated or insufficient to adequately correct dehydration
 - Appropriate intravenous treatment (eg, fluids) does not readily correct dehydration.

References

1. Padilipsky P, White W. Pediatric infectious diarrheal disease and dehydration. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:2151-2168.e3.
2. Mount DB. Fluid and electrolyte disturbances. In: Loscalzo J, Fauci A, Kasper D, Hauser S, Longo D, Jameson JL, editors. Harrison's Principles of Internal Medicine. 21st ed. McGraw Hill Education; 2022:338-356.
3. Greenbaum LA. Deficit therapy. In: Kliegman RM, St. Geme JW, Blum NJ, Shah SS, Tasker RC, Wilson KM, editors. Nelson Textbook of Pediatrics. 21st ed. Philadelphia, PA: Elsevier; 2020:429-432.e1.
4. Greenbaum LA. Maintenance and replacement therapy. In: Kliegman RM, St. Geme JW, Blum NJ, Shah SS, Tasker RC, Wilson KM, editors. Nelson Textbook of Pediatrics. 21st ed. Philadelphia, PA: Elsevier; 2020:425-428.e1.

Discharge planning

- Discharge planning includes **ALL** of the following(1)(2)(3)(4)(5):
 - Identification of needed internal and external resources and services (eg, medical/care providers, care manager, housing, transportation, financial support)
 - Patient, family, guardian, and other supports involved in plan development as appropriate
 - Communication of current information to next level of care providers
 - Identification of contacts and their responsibilities at next level of care

References

1. Sowers WE, Rohland B. American Association of Community Psychiatrists' principles for managing transitions in behavioral health services. Psychiatric Services 2004;55(11):1271-5.
2. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]

3. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
4. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
5. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.

Evaluation completed and reviewed

- Completion of appropriate evaluation includes **ALL** of the following(1)(2)(3):
 - Psychiatric history and evaluation
 - Mental status examination
 - Evaluation of precipitants
 - Observation and symptom assessment completed or not needed
 - Physical examination completed or not indicated for level of care

References

1. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.
2. Egger HL, Pataki CS. Psychiatric assessment of preschool children. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:3409-3413.
3. Shain B, Committee on Adolescence. Suicide and suicide attempts in adolescents. Pediatrics 2016;138(1):e20161420. DOI: 10.1542/peds.2016-1420. (Reaffirmed 2023 Jul)

Harm

- Harm to self or another is considered serious if it has a substantial likelihood of causing death, disability, or major disfigurement.(1)(2)(3)(4)(5)

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
3. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
4. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
5. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.

Hemodynamic instability

- Hemodynamic instability, as indicated by **1 or more** of the following(1)(2)(3)(4)(5)(6):
 - Vital sign abnormality not readily corrected by appropriate treatment, as indicated by **1 or more** of the following[A]:
 - Tachycardia that persists despite appropriate treatment (eg, volume repletion, treatment of pain, treatment of underlying cause)
 - Hypotension that persists despite appropriate treatment (eg, volume repletion, treatment of underlying cause)
 - Orthostatic hypotension that persists despite appropriate treatment (eg, volume repletion)
 - Hypotension that is severe, as indicated by **ALL** of the following:
 - Hypotension
 - Severe hypotension, as indicated by **1 or more** of the following:
 - Lactate of 2.0 mmol/L (18 mg/dL) or more secondary to hypotension (ie, hypoperfusion)[B](3)(6)
 - Metabolic acidosis (arterial or venous[C] pH less than 7.35) not otherwise explained
 - Mean arterial pressure[A][D] less than 65 mm Hg
 - IV inotropic or vasopressor medication required to maintain adequate blood pressure or perfusion

References

1. Puskarich MA, Jones AE. Shock. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:34-41.e1.
2. Lewis J, Patel B. Shock. In: Gershel JC, Rauch DA, editors. Caring for the Hospitalized Child: A Handbook of Inpatient Pediatrics. 3rd ed. Elk Grove Village, IL: American Academy of Pediatrics; 2023:107-114.
3. Singer M, et al. The Third International Consensus definitions for sepsis and septic shock (Sepsis-3). Journal of the American Medical Association 2016;315(8):801-810. DOI: 10.1001/jama.2016.0287.
4. Turner DA, Cheifetz IM. Shock. In: Kliegman RM, St. Geme JW, Blum NJ, Shah SS, Tasker RC, Wilson KM, editors. Nelson Textbook of Pediatrics. 21st ed. Philadelphia, PA: Elsevier; 2020:572-583.e1.
5. Nyhan A. Cardiology. In: Kleinman K, McDaniel L, Molloy M, editors. The Harriet Lane Handbook: A Manual for Pediatric House Officers. 22nd ed. 202: Elsevier; 2021:145-188.e4.
6. Naidu SS, et al. SCAI SHOCK stage classification expert consensus update: a review and incorporation of validation studies: this statement was endorsed by the American College of Cardiology (ACC), American College of Emergency Physicians (ACEP), American Heart Association (AHA), European Society of Cardiology (ESC) Association for Acute Cardiovascular Care (ACVC), International Society for Heart and Lung Transplantation (ISHLT), Society of Critical Care Medicine (SCCM), and Society of Thoracic Surgeons (STS) in December 2021. Journal of the American College of Cardiology 2022;Online. DOI: 10.1016/j.jacc.2022.01.018.
7. Wardi G, Brice J, Correia M, Liu D, Self M, Tainter C. Demystifying lactate in the emergency department. Annals of Emergency Medicine 2020;75(2):287-298. DOI: 10.1016/j.annemergmed.2019.06.027.
8. Levitt DG, Levitt JE, Levitt MD. Quantitative assessment of blood lactate in shock: measure of hypoxia or beneficial energy source. BioMed Research International 2020;2020:2608318. DOI: 10.1155/2020/2608318.
9. Bakker J, Postelnicu R, Mukherjee V. Lactate: where are we now? Critical Care Clinics 2020;36(1):115-124. DOI: 10.1016/j.ccc.2019.08.009.
10. Kraut JA, Madias NE. Lactic acidosis. New England Journal of Medicine 2014;371(24):2309-2319. DOI: 10.1056/NEJMra1309483.
11. Olivieri L, Chasm R. Diabetic ketoacidosis in the pediatric emergency department. Emergency Medicine Clinics of North America 2013;31(3):755-773. DOI: 10.1016/j.emc.2013.05.004.
12. Maloney GE, Glauser JM. Diabetes mellitus and disorders of glucose homeostasis. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:1543-1558.e2.
13. Arnold TD, Miller M, van Wessel KP, Evans JA, Balogh ZJ. Base deficit from the first peripheral venous sample: a surrogate for arterial base deficit in the trauma bay. Journal of Trauma 2011;71(4):793-7; discussion 797. DOI: 10.1097/TA.0b013e31822ad694.
14. Malinoski DJ, Todd SR, Slone S, Mullins RJ, Schreiber MA. Correlation of central venous and arterial blood gas measurements in mechanically ventilated trauma patients. Archives of Surgery 2005;140(11):1122-5. DOI: 10.1001/archsurg.140.11.1122.
15. Zakrisson T, McFarlan A, Wu YY, Keshet I, Nathens A. Venous and arterial base deficits: do these agree in occult shock and in the elderly? A Bland-Altman analysis. Journal of Trauma and Acute Care Surgery 2013;74(3):936-9. DOI: 10.1097/TA.0b013e318282747a.

Footnotes

- A. Criteria based upon clinician-acquired numeric values (eg, vital signs, oxygen saturation) should be used if they are accurate reflections of the patient's condition. Transitory findings (eg, abnormal only upon initial emergency department intake or only one time out of multiple readings) that rapidly improve with no or minimal treatment usually do not reflect disease severity or risk for deterioration. This does not imply that an initial or one-time reading cannot ever be applicable. The goal is to separate erroneous or incidental findings from those that truly represent the patient's clinical picture.
- B. There are numerous causes of an elevated lactate level. The most common are cardiogenic or hypovolemic shock, severe heart failure, severe trauma, or sepsis. However, there are also other etiologies to consider such as vigorous exercise, seizures, liver disease, or medication use (eg, metformin, beta2-agonists); therefore, interpretation of elevated lactate levels requires consideration of the clinical context (eg, well-appearing post exercise vs hypotensive). The severity and persistence of the elevation can sometimes be helpful in this differentiation. In most instances of lactic acidosis, blood pH is less than 7.35, with a serum bicarbonate level 20 mEq/L (mmol/L) or lower. However, a coexisting respiratory or metabolic alkalosis can mask these findings.(6)(7)(8)(9)(10)
- C. Analyses have concluded that venous and arterial pH measurements are highly correlated, with small differences between them that are usually not clinically significant.(11)(12)(13)(14)(15) If a mixed acid-base disorder is suspected, an arterial blood gas is indicated. (12)
- D. The mean arterial pressure takes into account both systolic and diastolic blood pressure readings and is calculated as mean arterial pressure (MAP) equals 1/3 SBP + 2/3 DBP.(1)(4) A calculator is available at <https://www.mdcalc.com/mean-arterial-pressure-map>.

Hypotension

- Hypotension, as indicated by **ALL** of the following(1)(2)(3)(4):
 - Not patient baseline (eg, healthy adult with low SBP) or intentional therapeutic goal (eg, low SBP as treatment goal in heart failure)
 - Low blood pressure, as indicated by **1 or more** of the following:
 - SBP less than 90 mm Hg in adult or child 10 years or older[A]
 - Mean arterial pressure[A][B] less than 70 mm Hg in adult or child 10 years or older

- Decrease in baseline mean arterial pressure of 25% or more,[A][B] with significant signs or symptoms due to lower blood pressure (eg, near syncope, syncope, chest pain)
- SBP less than sum of 70 mm Hg plus twice patient's age in years in child 1 to 9 years of age[A]
- SBP less than 70 mm Hg in infant 1 to 11 months of age[A]

References

1. Jones D, Di Francesco L. Hypotension. In: McKean SC, Ross JJ, Dressler DD, Scheurer DB, editors. Principles and Practice of Hospital Medicine. 2nd ed. New York, NY: McGraw-Hill Education; 2017:657-64.
2. Massaro AF. Approach to the patient with shock. In: Loscalzo J, Fauci A, Kasper D, Hauser S, Longo D, Jameson JL, editors. Harrison's Principles of Internal Medicine. 21st ed. McGraw Hill Education; 2022:2235-2241.
3. Horeczko T. Pediatric cardiac disorders. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:2109-2131.e1.
4. Singh S, Holmes JF. Pediatric trauma. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:2052-2066.e3.

Footnotes

- A. Criteria based upon clinician acquired numeric values (eg, vital signs, oxygen saturation) should be used if they are accurate reflections of the patient's condition. Transitory findings (eg, abnormal only upon initial emergency department intake or only one time out of multiple readings) that rapidly improve with no or minimal treatment usually do not reflect disease severity or risk for deterioration. This does not imply that an initial or one-time reading cannot ever be applicable. The goal is to separate erroneous or incidental findings from those that truly represent the patient's clinical picture.
- B. The mean arterial pressure takes into account both systolic and diastolic blood pressure readings and is calculated as Mean Arterial Pressure (MAP) = $1/3 \text{ SBP} + 2/3 \text{ DBP}$. A calculator is available at <https://www.mdcalc.com/mean-arterial-pressure-map>.

Hypoxemia

- Hypoxemia, as indicated by **1 or more** of the following(1):
 - Patient without baseline need for supplemental oxygen with oxygen saturation (SaO₂) of less than 90% or arterial blood gas partial pressure of oxygen (PO₂) of less than 60 mm Hg (8.0 kPa) on room air[A][B]
 - Patient without baseline need for supplemental oxygen who now requires supplemental oxygen to keep SaO₂ greater than 89% or PO₂ greater than 59 mm Hg (7.9 kPa)[A]
 - Patient with baseline need for supplemental oxygen who now requires increased supplemental oxygen to maintain oxygenation at baseline or acceptable level

References

1. Dezube R, Lechtzin N. Hypoxia. In: McKean SC, Ross JJ, Dressler DD, Scheurer DB, editors. Principles and Practice of Hospital Medicine. 2nd ed. New York, NY: McGraw-Hill Education; 2017:669-675.
2. Jamali H, et al. Racial disparity in oxygen saturation measurements by pulse oximetry: evidence and implications. Annals of the American Thoracic Society 2022;19(12):1951-1964. DOI: 10.1513/AnnalsATS.202203-270CME.
3. Rojas-Camayo J, et al. Reference values for oxygen saturation from sea level to the highest human habitation in the Andes in acclimatised persons. Thorax 2018;73(8):776-778. DOI: 10.1136/thoraxjnl-2017-210598.

Footnotes

- A. Pulse oximetry (SpO₂) may underestimate arterial saturation (SaO₂), especially in people with dark skin pigmentation, thick skin, cold body temperature, or who are wearing colored nail polish, and thus may not accurately detect hypoxemia.(2) Where appropriate, pulse oximetry should be measured after warming fingers or removing nail polish. Pulse oximetry results should be assessed within the context of the patient's clinical presentation, and performance of an arterial blood gas considered for a more accurate assessment of oxygenation if indicated.(2) Specific target values may require adjustment when care is delivered at high altitude.(1)(3)
- B. Criteria based upon clinician-acquired numeric values (eg, vital signs, oxygen saturation) should be used if they are accurate reflections of the patient's condition. Transitory findings (eg, abnormal only upon initial emergency department intake or only one time out of multiple readings) that rapidly improve with no or minimal treatment usually do not reflect disease severity or risk for deterioration. This does not imply that an initial or one-time reading cannot ever be applicable. The goal is to separate erroneous or incidental findings from those that truly represent the patient's clinical picture.

Medical needs absent or manageable/treatable at available lower level of care

- Medical needs absent or manageable/treatable at available lower level of care, as indicated by **ALL** of the following(1)(2)(3)(4)(5)(6):
 - Adverse medication effects absent or manageable/treatable
 - Medical comorbidity absent or manageable/treatable

- Medical complications absent or manageable/treatable (eg, complications of eating disorder)
- Substance-related disorder absent or manageable/treatable

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
3. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
4. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.
5. Matching multidimensional severity and level of function with type and intensity of service. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provenance SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:69-104.
6. Dimensional admission criteria and algorithm. In: Waller RC, et al., editors. The ASAM Criteria: Treatment Criteria for Addictive, Substance-Related, and Co-occurring Conditions, Volume 1: Adults. 4th ed. Hazelden Publishing; 2023.

Orthostatic hypotension

- Orthostatic hypotension,^{[A][B]} as indicated by **1 or more** of the following(1)(2)(3):
 - Fall in SBP of 20 mm Hg or more 1 to 3 minutes after patient sits or stands from recumbent position
 - Fall in DBP of 10 mm Hg or more 1 to 3 minutes after patient sits or stands from recumbent position

References

1. Shibao C, Lipsitz LA, Biaggioni I, American Society of Hypertension Writing Group. Evaluation and treatment of orthostatic hypotension. Journal of the American Society of Hypertension 2013 Jul-Aug;7(4):317-324. DOI: 10.1016/j.jash.2013.04.006.
2. Dalal AS, Van Hare GF. Syncope. In: Kliegman RM, St. Geme JW, Blum NJ, Shah SS, Tasker RC, Wilson KM, editors. Nelson Textbook of Pediatrics. 21st ed. Philadelphia, PA: Elsevier; 2020:566-571.e1.
3. Fang JC, O'Gara PT. History and physical examination: an evidence-based approach. In: Libby P, Bonow RO, Mann DL, Tomaselli GF, Bhatt DL, Solomon SD, editors. Braunwald's Heart Disease: A Textbook of Cardiovascular Medicine. 12th ed. Elsevier; 2022:123-140.

Footnotes

- A. Concomitant measurements of the heart rate are important to measure to help diagnose subtypes of orthostatic hypotension (eg, the lack of a compensatory increase in heart rate is typical of autonomic failure and an exaggerated tachycardia may be reflective of volume depletion). However, the heart rate is not a component of the definition of orthostatic hypotension which relies upon blood pressure alone.(1)(2)(3)
- B. Criteria based upon clinician acquired numeric values (eg, vital signs, oxygen saturation) should be used if they are accurate reflections of the patient's condition. Transitory findings (eg, abnormal only upon initial emergency department intake or only one time out of multiple readings) that rapidly improve with no or minimal treatment usually do not reflect disease severity or risk for deterioration. This does not imply that an initial or one-time reading cannot ever be applicable. The goal is to separate erroneous or incidental findings from those that truly represent the patient's clinical picture.

Purging, bingeing, and other problem behavior absent or manageable/treatable at available lower level of care

- Purging, bingeing, and other problem behavior absent or manageable/treatable at available lower level of care, as indicated by **ALL** of the following:
 - Purging (eg, self-induced vomiting), bingeing, and other problem behavior (eg, excessive exercise) absent or manageable/treatable
 - Patient able to function and participate at available lower level of care

Respiratory distress

- Respiratory distress, as indicated by **ALL** of the following(1)(2):
 - Patient with **1 or more** of the following:
 - Dyspnea (difficulty breathing)
 - Tachypnea

- Abnormal breathing pattern (eg, chest retractions)
- Other evidence of difficulty breathing
- Evidence of respiratory compromise, as indicated by **1 or more** of the following:
 - Hypoxemia
 - Altered mental status
 - Other evidence of respiratory compromise (eg, respiratory acidosis)

References

1. Braithwaite SA, Wessel AL. Dyspnea. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:194-201.e2.
2. Naureckas ET, Solway J. Disturbances of respiratory function. In: Loscalzo J, Fauci A, Kasper D, Hauser S, Longo D, Jameson JL, editors. Harrison's Principles of Internal Medicine. 21st ed. McGraw Hill Education; 2022:2133-2139.

Risk status acceptable

- Risk status acceptable, as indicated by **ALL** of the following(1)(2)(3)(4)(5)(6):
 - Danger to self or others manageable/treatable, as indicated by **1 or more** of the following:
 - Absence of Thoughts of suicide, homicide, or serious Harm to self or to another
 - Thoughts of suicide, homicide, or serious Harm to self or to another present but manageable/treatable at available lower level of care
 - Patient and supports understand follow-up treatment and crisis plan.
 - Provider and supports are sufficiently available at lower level of care.
 - Patient, as appropriate, can participate as needed in monitoring at available lower level of care.

References

1. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
2. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
3. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
4. Early Childhood Service Intensity Instrument (ECSII) for Infants, Toddlers, and Preschool-aged Children Ages 0-5 version 1. American Academy of Child and Adolescent Psychiatry 2009 Sep.
5. Matching multidimensional severity and level of function with type and intensity of service. In: Mee-Lee D, Shulman GD, Fishman MJ, Gastfriend DR, Miller MM, Provence SM, editors. ASAM Criteria Treatment Criteria for Addictive, Substance-Related, and Co-Occurring Conditions. 3rd ed. Carson City, NV: The Change Companies; 2013:69-104.
6. Dimensional admission criteria and algorithm. In: Waller RC, et al., editors. The ASAM Criteria: Treatment Criteria for Addictive, Substance-Related, and Co-occurring Conditions, Volume 1: Adults. 4th ed. Hazelden Publishing; 2023.

Social Determinants of Health Assessment

- Risk of poor health outcomes may be increased by the presence of **1 or more** of the following social determinants of health(1)(2)(3)(4):
 - Housing insecurity, as indicated by **1 or more** of the following:
 - Individual or caregiver's current living situation is **1 or more** of the following(5):
 - Does not have own housing (eg, staying in a hotel, shelter, or with others)
 - Has own housing (eg, house, apartment), but at risk of losing it in the future (ie, behind on rent or mortgage)
 - Has own housing (eg, house, apartment), but has lived in 3 or more places in past year
 - Current housing has **1 or more** of the following:
 - Electrical appliances (eg, stove, refrigerator) not working or unavailable
 - Insufficient heating or cooling
 - Insufficient ventilation
 - Lead paint or pipes
 - Mold
 - Pests (eg, bugs) or rodents
 - Smoke detectors not working or unavailable
 - Food insecurity, as indicated by **1 or more** of the following(6):
 - In the past year, individual or caregiver ran out of food and did not have money to buy more food.
 - In the past year, individual or caregiver worried that they would run out of food before they received money to buy more food.

- Insufficient transportation, as indicated by **1 or more** of the following(7):
 - In the past year, individual or caregiver missed medical appointments or could not get medications due to lack of transportation.
 - In the past year, individual or caregiver missed nonmedical activities, work, or could not get things needed for daily living due to lack of transportation.
- Insufficient utilities, as indicated by **1 or more** of the following(8):
 - Utilities (eg, electricity, water, gas, or oil) are currently shut off or unavailable.
 - In the past year, electric, water, gas, or oil company threatened to shut off services.
- Personal safety risk, as indicated by **2 or more** of the following(6):
 - Individual is sometimes or frequently physically hurt by another person (including family member).
 - Individual is sometimes or frequently insulted or talked down to by another person (including family member).
 - Individual is sometimes or frequently threatened with physical harm by another person (including family member).
 - Individual is sometimes or frequently screamed or cursed at by another person (including family member).
- Insufficient dependent care, as indicated by **1 or more** of the following:
 - In the past year, individual or caregiver was unable to work due to lack of dependent care.
 - In the past year, individual or caregiver was unable to work more (additional) hours due to lack of dependent care.
 - In the past year, individual or caregiver missed medical appointments or could not get medications due to lack of dependent care.
 - In the past year, individual or caregiver missed nonmedical activities (eg, school, church, social activity) due to lack of dependent care.
- Depression risk, as indicated by **ALL** of the following:
 - In the past 2 weeks, individual had little interest or pleasure in normal activities on at least several days.
 - In the past 2 weeks, individual felt down, depressed, or hopeless on at least several days.

References

1. Scanlon A, Reinisch C. Social determinants of health. In: Harding MM, Kwong J, Hagler D, Reinisch C, editors. *Lewis's Medical-Surgical Nursing: Assessment and Management of Clinical Problems*. 12th ed. St. Louis, MO: Mosby; 2023:18-20.
2. Moen M, Storr C, German D, Friedmann E, Johantgen M. A review of tools to screen for social determinants of health in the United States: a practice brief. *Population Health Management* 2020;23(6):422-429. DOI: 10.1089/pop.2019.0158.
3. Daniel-Robinson L, Moore JE. Innovation and Opportunities to Address Social Determinants of Health in Medicaid Managed Care. [Internet] Institute for Medicaid Innovation. 2019 Jan Accessed at: <https://medicaidinnovation.org/emerging-topics/>. [accessed 2023 Oct 20]
4. Billioux A, Verlander K, Anthony S, Alley D. Standardized Screening for Health-Related Social Needs in Clinical Settings: the Accountable Health Communities Screening Tool. [Internet] National Academy of Sciences. 2017 May Accessed at: <https://nam.edu/>. [accessed 2023 Sep 18]
5. Sandel M, et al. Unstable housing and caregiver and child health in renter families. *Pediatrics* 2018;14(2):e20172199. DOI: 10.1542/peds.2017-2199.
6. Children's HealthWatch Survey. Screening Instrument [Internet] Children's HealthWatch. 2020 Sep Accessed at: <https://childrenshealthwatch.org/>. [accessed 2023 Sep 11]
7. PRAPARE®: Protocol for Responding to and Assessing Patient Assets, Risks, and Experiences Screening Tool. [Internet] Association of Asian Pacific Community Health Organizations (AAPCHO) and National Association of Community Health Centers (NACHC). 2016 Sep Accessed at: <https://prapare.org/the-prapare-screening-tool/>. [accessed 2023 Sep 21]
8. Cook JT, et al. A brief indicator of household energy security: associations with food security, child health, and child development in US infants and toddlers. *Pediatrics* 2008;122(4):e867-75. DOI: 10.1542/peds.2008-0286.

Tachycardia

- Tachycardia,^{[A][B]} as indicated by **1 or more** of the following(1)(2):
 - Heart rate greater than 100 beats per minute in adult or child age 6 years or older
 - Heart rate greater than 115 beats per minute in child 3 to 5 years of age
 - Heart rate greater than 125 beats per minute in child 1 or 2 years of age
 - Heart rate greater than 130 beats per minute in infant 6 to 11 months of age
 - Heart rate greater than 150 beats per minute in infant 3 to 5 months of age
 - Heart rate greater than 160 beats per minute in infant 1 or 2 months of age

References

1. Southmayd GL. Tachycardia. In: McKean SC, Ross JJ, Dressler DD, Scheurer DB, editors. *Principles and Practice of Hospital Medicine*. 2nd ed. New York, NY: McGraw-Hill Education; 2017:729-739.
2. Pediatric parameters and equipment. In: Kleinman K, McDaniel L, Molloy M, editors. *The Harriet Lane Handbook: A Manual for Pediatric House Officers*. 22nd ed. 202: Elsevier; 2021:frontpiece tables.

Footnotes

- A. Criteria based upon clinician acquired numeric values (eg, vital signs, oxygen saturation) should be used if they are accurate reflections of the patient's condition. Transitory findings (eg, abnormal only upon initial emergency department intake or only one time out of multiple readings) that rapidly improve with no or minimal treatment usually do not reflect disease severity or risk for deterioration. This does not imply that an initial or one-time reading cannot ever be applicable. The goal is to separate erroneous or incidental findings from those that truly represent the patient's clinical picture.
- B. Interpretation of heart rate requires clinical judgment and consideration of several patient-specific factors, such as the patient's baseline heart rate, medications, and clinical impact. For example, an elderly patient on a beta-blocker medication with a baseline resting heart rate of 60 beats per minute may be clinically tachycardic at a heart rate of 94 beats per minute. Likewise, a patient who is upset, in pain, or nervous in the emergency department with a heart rate of 106 beats per minute may meet the technical definition of tachycardia, but this tachycardia (absent associated findings such as chest pain or hypotension) may not be clinically important. The numeric values included in this definition are provided to allow for consistency in terms of a technical definition of the term tachycardia. Whether a heart rate above or below the technical threshold is clinically meaningful is a matter of persistence, context, and clinical judgment.

Tachypnea

- Tachypnea,^{[A][B]} as indicated by respiratory rate of **1 or more** of the following⁽²⁾⁽³⁾:
 - Greater than 18 breaths per minute in adult or child 13 years of age or older^[A]
 - Greater than 22 breaths per minute in child 6 to 12 years of age^[A]
 - Greater than 25 breaths per minute in child 3 to 5 years of age^[A]
 - Greater than 30 breaths per minute in child 1 or 2 years of age^[A]
 - Greater than 40 breaths per minute in infant 6 to 11 months of age^[A]
 - Greater than 45 breaths per minute in infant 3 to 5 months of age^[A]
 - Greater than 60 breaths per minute in infant 1 or 2 months of age^[A]

References

1. Schriger DL. Approach to the patient with abnormal vital signs. In: Goldman L, Schafer AI, editors. Goldman-Cecil Medicine. 26th ed. Elsevier; 2020:28-31.e2.
2. Braithwaite SA, Wessel AL. Dyspnea. In: Walls RM, editor. Rosen's Emergency Medicine: Concepts and Clinical Practice. 10th ed. Philadelphia, PA 19103-2899: Elsevier; 2023:194-201.e2.
3. Pediatric parameters and equipment. In: Kleinman K, McDaniel L, Molloy M, editors. The Harriet Lane Handbook: A Manual for Pediatric House Officers. 22nd ed. 202: Elsevier; 2021:frontpiece tables.

Footnotes

- A. Criteria based upon clinician acquired numeric values (eg, vital signs, oxygen saturation) should be used if they are accurate reflections of the patient's condition. Transitory findings (eg, abnormal only upon initial emergency department intake or only one time out of multiple readings) that rapidly improve with no or minimal treatment usually do not reflect disease severity or risk for deterioration. This does not imply that an initial or one-time reading cannot ever be applicable. The goal is to separate erroneous or incidental findings from those that truly represent the patient's clinical picture.
- B. Interpretation of respiratory rate requires clinical judgment and consideration of several patient-specific factors, such as the patient's baseline respiratory rate and whether the respiratory rate is indicative of significant underlying respiratory or other pathology. A mildly increased respiratory rate (eg, 20 to 22 in an adult), absent other indications of serious illness (eg, hypoxemia, metabolic acidosis, decreased tidal volume, pain), may be transitory and not clinically important. The numeric values included in this definition are provided to allow for consistency in terms of a technical definition of the term tachypnea. Whether a respiratory rate above the technical threshold is clinically meaningful is a matter of persistence, context, and clinical judgment. In addition, the measurement of the respiratory rate is subject to error. A medical textbook states that because there can be significant breath to breath variation in respiratory rate, the rate should be assessed for at least 30 seconds, preferably for a full minute.⁽¹⁾ This same textbook also states that new technologies designed to measure the respiratory rate have not proved to be clinically useful.⁽¹⁾

Telehealth services

- Telehealth services involve a remote exchange of health data between patient and provider. The data are exchanged in real time, using equipment that may send data via the internet, audio interaction, or video conferencing. The types of services delivered could include medical or rehabilitation examination, assessment, condition management, care coordination, or education.⁽¹⁾⁽²⁾

References

1. Matsumoto ME, Wilske GC, Tapia R. Innovative approaches to delivering telehealth. Physical Medicine and Rehabilitation Clinics 2021;32(2):451-465. DOI: 10.1016/j.pmr.2020.12.008.
2. Gies CE. Health-related quality of life. In: Larsen PD, editor. Lubkin's Chronic Illness: Impact and Intervention. 11th ed. Jones & Bartlett Learning; 2022:49-86.

Thoughts of suicide

- Thoughts of suicide or suicidal ideations are defined as "thoughts of serving as the agent of one's own death," to be distinguished from thoughts of death that do not involve actively bringing death about.(1)(2)

References

1. Sudak HS. Suicide treatment. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:2618-2622.
2. APA Work Group on Psychiatric Evaluation. The American Psychiatric Association practice Guidelines for the Psychiatric Evaluation of Adults. 3rd edition [Internet] American Psychiatric Association. 2016 Accessed at: <https://psychiatryonline.org/>. [accessed 2023 Sep 21] DOI: 10.1176/appi.books.9780890426760.

Treatment plan with goals and progress measurement in place

- A comprehensive treatment plan includes **ALL** of the following(1)(2)(3)(4)(5)(6):
 - Definition of individualized short-term and long-term patient goals with measurable progress indicators
 - Description of medication, psychotherapy, and other social interventions designed to assist progress toward goals
 - Plan for monitoring and measuring progress toward goals
 - Participation of patient, family, guardian, and other supports as appropriate

References

1. Geller JL. The role of the hospital in the care of persons with mental illness. In: Sadock BJ, Sadock VA, Ruiz P, editors. Kaplan and Sadock's Comprehensive Textbook of Psychiatry. 10th ed. Philadelphia, PA: Lippincott Williams & Wilkins; 2017:4325-4346.
2. American Association of Community Psychiatrists. Level of Care Utilization System for Psychiatric and Addiction Services (LOCUS). Adult version 20 [Internet] American Association of Community Psychiatrists. 2016 Dec Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 11]
3. American Association of Community Psychiatrists. CALOCUS/CASII: Child and Adolescent Level of Care Utilization System. Child and Adolescent Version 20 [Internet] American Association of Community Psychiatrists. 2019 Jul Accessed at: <https://www.communitypsychiatry.org/keystone-programs/locus>. [accessed 2023 Sep 08]
4. CASII. Child and Adolescent Service Intensity Instrument [Internet] American Academy of Child and Adolescent Psychiatry. Accessed at: https://www.aacap.org/aacap/Member_Resources/Practice_Information/CASII.aspx. [accessed 2023 Sep 25]
5. Child & Adolescent Service Intensity Instrument (CASII) User's Manual version 4.0. American Academy of Child and Adolescent Psychiatry 2014 Oct.
6. Rosser J, Michael S. Partial Hospitalization Programs and Intensive Outpatient Programs. 2021 AABH Standards and Guidelines [Internet] Association for Ambulatory Behavioral Healthcare. 2021 Accessed at: <https://aabh.org/>. [accessed 2023 Sep 12]

Weight status acceptable

- Weight status acceptable, as indicated by **1 or more** of the following(1):
 - Weight is increasing at rate of at least 1 lb (0.5 kg) per week in residential setting.
 - Weight is increasing at rate of at least 2 lb (0.9 kg) per week in hospital setting.
 - Weight is stable, and reason for admission was not absence of recent weight gain.

References

1. Crone C, et al. The American Psychiatric Association Practice Guideline for the Treatment of Patients With Eating Disorders. 4th ed [Internet] American Psychiatric Association. 2023 Feb Accessed at: http://www.psych.org/psych_pract/treatg/pg/prac_guide.cfm. [accessed 2023 Sep 19]

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